

Stress Incontinence Surgery Without Mesh

Patient Information · Burch colposuspension and the fascial sling

WARWIKI

Stress urinary incontinence is leaking urine when you cough, laugh, lift or exercise. The most common surgery is a mesh mid-urethral sling, which has its own handout. Two well-studied operations support the urethra **without synthetic mesh**: the **Burch colposuspension**, which uses stitches, and the **fascial sling**, which uses a strip of your own tissue.

About These Operations

- **Burch colposuspension.** Through a low “bikini” cut, or through small keyhole or robotic cuts, stitches are placed in the tissue on each side of the urethra. They are tied to a strong ligament behind the pubic bone to support the urethra.
- **Fascial (pubovaginal) sling.** A strip of **fascia**, the tough layer over the belly muscles or on the side of the thigh, is taken through a separate cut. It is placed under the urethra through a small cut in the vagina, like a hammock. Its ends are fixed above the pubic bone.

Benefits and Risks in Numbers

about
2 in 3

had no stress leaking 2 years after a fascial sling, compared with 1 in 2 after a Burch

about
8 in 10

were satisfied 5 years after a fascial sling, compared with 7 in 10 after a Burch

about
1 in 5

had trouble emptying after a fascial sling in one study. Half got better with self-catheterization for a while

The first two boxes come from one large trial that compared the two operations. Your own risk may differ; ask your team.

How the Choices Compare

	Burch	Fascial sling	Mesh sling
Support from	Stitches	Your own tissue	Synthetic mesh tape
Cuts	Low belly, or keyhole	Low belly or thigh, plus vagina	Small vaginal cut and tiny skin cuts
Stress leaking	Good control	Better than Burch	Similar to a fascial sling
Trouble emptying	Less common	More common	Can happen
Also know	Later bulging of the back vaginal wall is more common	A second wound to heal; the operation takes longer	Mesh can show through the vaginal wall

Another choice without mesh or a cut: urethral bulking. A gel (such as Bulkamid) is injected into the urethra wall, often in the office. Quick and low risk, but it works less well than surgery and often needs a top-up.

Learn the Terms

Stress incontinence

Leaking urine with pressure, such as coughing, laughing or lifting.

Urethra

The tube that carries urine out of the body.

Burch colposuspension

Stitches beside the urethra, tied to a ligament behind the pubic bone.

Fascia

A tough, thin layer of tissue over muscle. A strip can be moved to make a sling.

Fascial (pubovaginal) sling

A sling made from your own fascia, placed under the urethra.

Self-catheterization

Passing a thin tube into the bladder yourself to empty it.

Which one is right for me? No one operation is best for everyone. A Burch may suit you if you want to avoid mesh and a belly operation is already planned. A fascial sling may suit you if your urethra does not move much, if earlier surgery did not work, or if another repair near the urethra is done at the same time.

Before you leave

You urinate to show your bladder empties. If not, a catheter goes home for a short time.

First weeks

Soreness at the cuts. Walking is good. Your team tells you what to avoid.

About 6 weeks

The cuts are healed. Ask your team about sex and tampons.

Within 6 months

A visit to check leaking, emptying and how the cuts healed.

How to Get Ready

- Follow your surgery instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** checks for infection. You may also have bladder testing and a check of how well you empty.
- Tell your team if you may want to have a baby later or are planning weight-loss surgery, or if you have had pelvic surgery or radiation.
- Arrange a ride home.

What Happens

- 1 You are asleep or numb from the waist down, and you get antibiotics.
- 2 **Burch:** stitches are placed beside the urethra and tied behind the pubic bone. **Fascial sling:** a strip of fascia is taken, placed under the urethra and fixed above the pubic bone.
- 3 The surgeon looks inside your bladder with a thin camera to check for injury.
- 4 A catheter drains your bladder for a short time.

After Surgery

- Trouble emptying is more likely after a fascial sling. It often settles. If it lasts, self-catheterization or loosening the sling are options.
- Urgency leaking may continue or start. Tell your team; it is treated in other ways.
- Bulging of the back vaginal wall (prolapse) is more common later after a Burch. Tell your team about new bulging or pressure.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or fluid from a cut, or pain that gets worse
- A weak stream, or a feeling that your bladder is not emptying

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You have heavy bleeding that soaks a pad an hour
- You cannot reach the office and are worried

Three Things to Remember

1. A Burch and a fascial sling treat stress leaking without synthetic mesh.
2. A fascial sling controls stress leaking better than a Burch, but trouble emptying is more common.
3. Urgency leaking can continue after either. Tell your team.